BEYOND FINANCIAL INCENTIVES: LESSONS ON RURAL PHYSICIAN RECRUITMENT

THE PROBLEM

In Canada, rural and remote communities account for approximately 18% of the population but are served by about 8% of physicians, reflecting a longstanding structural imbalance in the health workforce. This project responds to a critical policy question for rural and Northern communities: *can financial incentives meaningfully improve the distribution of family physicians*?

ARTICLE TITLE

Do Financial Incentives Affect Rural Physician Supply? Empirical Evidence from a Canadian Experiment

BACKGROUND

The project aims to examine whether financial incentives can mitigate the rural physician shortage in Canada.

Using a natural experiment method leveredging Saskatchewan's Rural Physician Incentive Program (RPIP), the evaluate the impact of direct annual payments for rural practice on the rural–urban distribution of family physicians

METHODOLOGY

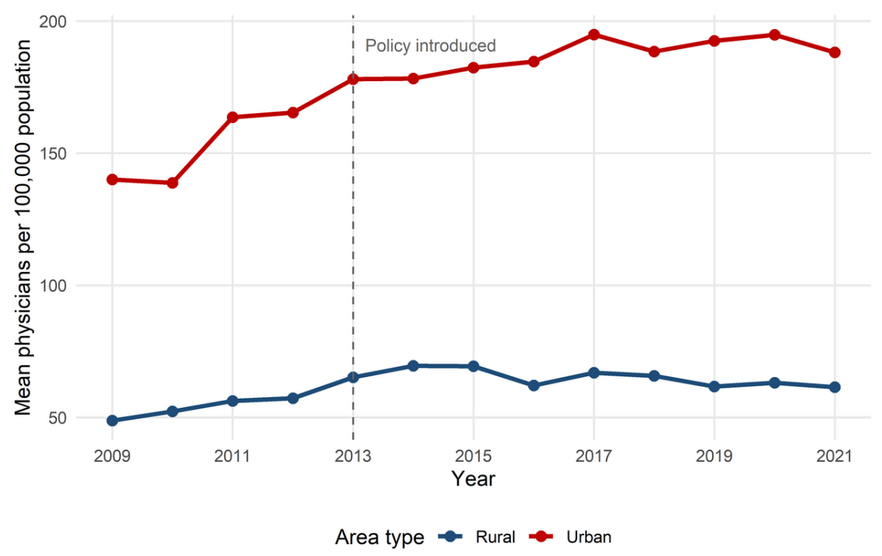
Target Program: Rural Physician Incentive Program (RPIP)

Aim: To assess whether the program improved the distribution of physicians between rural and urban areas.

Data: (1) Administrative physician data from CIHI and (2) Population data from Statistics Canada

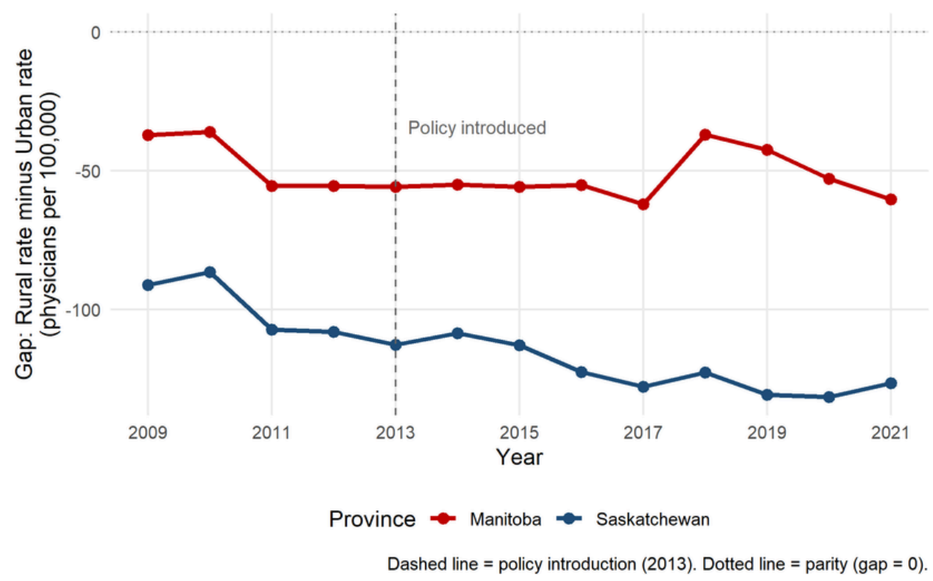
Year: From 2009 to 2021

Figure 1: Physician rates by area type: Mixed Saskatchewan regions



Dashed line indicates policy introduction (2013).

Figure 2: Rural–urban physician rate gap by province: Mixed health region



Dashed line = policy introduction (2013). Dotted line = parity (gap = 0).

FINDINGS

- Rural physician rates remained persistently below urban rates
- We found no statistically robust evidence that the incentive program narrowed the rural–urban physician supply gap.
- The study suggests that stand-alone financial payments may be insufficient when deeper structural barriers such as, professional isolation, family employment opportunities, infrastructure, and rural practice support, remain unresolved.

Journal: Social Science & Medicine
Year: 2026

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Project Team: Dr. Tamanna Rimi, Dr. Ahmed Aziz, Quazi Nazmus Sakib